

360 mm

CARBETOCIN 100 µg/ml solution for injection

1. NAME OF THE MEDICINAL PRODUCT

Pharmidea Carbetocin 100 µg/ml solution for injection

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each millilitre (ml) of solution contains 100 micrograms of carbetocin.

For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Solution for injection.

Clear, colourless liquid, free of visible particles.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Pharmidea Carbetocin is indicated for the prevention of postpartum haemorrhage due to uterine atony.

4.2 Posology and method of administration

Caesarean section under epidural or spinal anaesthesia:

Withdraw 1 ml of Pharmidea Carbetocin containing 100 micrograms carbetocin and administer only by intravenous injection, under adequate medical supervision in a hospital.

Vaginal delivery:

Withdraw 1mg of Pharmidea Carbetocin containing 100 micrograms carbetocin and administer by intravenous injection or intramuscular injection, under adequate medical supervision in a hospital.

Method of administration

For intravenous or intramuscular administration. Carbetocin must only be administered after delivery of the infant, and as soon as possible after delivery, preferably before the delivery of the placenta. For intravenous administration carbetocin must be administered slowly, over 1 minute. Pharmidea Carbetocin is intended for single use only. No further doses of carbetocin should be administered.

Peadiatric population

There is no relevant use of carbetocin in children below 12 years of age.

The safety and efficacy of carbetocin in adolescents has not yet been established.

4.3 Contraindications

- Hypersensitivity to the active substance or to any of the excipients or oxytocin.
- During pregnancy and labour before delivery of the infant.
- Carbetocin must not be used for the induction of labour.

- Pre-eclampsia and eclampsia.
- Hepatic or renal disease.
- Serious cardiovascular disorders.
- Epilepsy.

4.4 Special warnings and precautions for use

Carbetocin is intended for use only at well-equipped specialist obstetrics units with experienced and qualified staff available at all times.

The use of carbetocin at any stage before delivery of the infant is not appropriate because its uterotonic activity persists for several hours after a single bolus injection. This is in marked contrast to the rapid reduction of effect observed after discontinuation of an oxytocin infusion.

In case of persistent uterine bleeding after administration of carbetocin, the cause must be determined. Consideration should be given to causes such as retained placental fragments, inadequate emptying or repair of the uterus, or disorders of blood coagulation.

Carbetocin is intended for single administration only, intramuscular or intravenous. It must be administered slowly over one minute. In case of persisting uterine hypotonia or atonia and the consequent excessive bleeding, additional therapy with oxytocin and/or ergometrine should be considered. There are no data on additional doses of carbetocin or on the use of carbetocin following persisting uterine atony after oxytocin.

Carbetocin possess some antidiuretic activity (vasopressin activity: < 0,025 IU/vial) and therefore the possibility of hyponatraemia cannot be excluded, particularly in patients also receiving large volumes of intravenous fluids. The early signs of drowsiness, listlessness and headache should be recognised to prevent convulsions and coma.

In general, carbetocin should be used cautiously in the presence of migraine, asthma and cardiovascular disease or any state in which a rapid addition to extracellular water may produce hazard for an already overburdened system. The decision of administering carbetocin can be made by the physician after carefully weighing the potential benefit carbetocin may provide in these particular cases.

No data is available on the use of carbetocin in patients with eclampsia. Patients with eclampsia and pre-eclampsia should be carefully monitored.

Specific studies have not been undertaken in gestational diabetes mellitus.

This medicinal product contains less than 1 mmol sodium (23 mg) per dose.

4.5 Interaction with other medicinal products and other forms of interaction

No drug interactions have been identified when carbetocin has been administered in association with a number of analgesics, spasmolytics and agents used for epidural or spinal anaesthesia. Specific interaction studies have not been undertaken. Since carbetocin is closely related in structure to oxytocin, the occurrence of interactions known to be associated with oxytocin cannot be excluded.

Severe hypertension has been reported when oxytocin was given 3 to 4 hours following prophylactic administration of a vasoconstrictor in conjunction with caudal-block anaesthesia.

During combination with ergot-alkaloids, such as methylergometrine, oxytocin and carbetocin may enhance the blood pressure enhancing effect of these agents. If oxytocin or methylergometrine are administered after carbetocin there may be a risk of cumulative exposure.

Since it has been found that prostaglandins potentiate the effect of oxytocin, it is expected that this can also occur with carbetocin. Therefore, it is not recommended that prostaglandins and carbetocin be used together. If they are concomitantly administered, the patient should be carefully monitored.

Some inhalation-anaesthetics, such as halothane and cyclopropane may enhance the hypotensive effect and weaken the effect of carbetocin on the uterus. Arrhythmias have been reported for oxytocin during concomitant use.

4.6 Fertility, pregnancy and lactation

Pregnancy

Carbetocin is contraindicated during pregnancy and must not be used for the induction of labour.

Breast-feeding

Small amounts of carbetocin pass from plasma into breast milk of nursing women. The small amounts transferred into colostrum or breast milk after a single injection of carbetocin, and subsequently ingested by the infant are assumed to be degraded by enzymes in the gut.

4.7 Effects on ability to drive and use machines

Not relevant.

4.8 Undesirable effects

The adverse events observed with carbetocin are of the same type and frequency as the adverse events observed with oxytocin when administered after caesarean section under spinal or epidural anaesthesia.

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The adverse effects in the table below are presented according to the MedDRA System Organ Class database and frequency convention.

| System Organ Class                                   | Very Common            | Common                   |
|------------------------------------------------------|------------------------|--------------------------|
| Blood and lymphatic system disorders                 |                        | Anaemia                  |
| Nervous system disorders                             | Headache, tremor       | Dizziness                |
| Vascular disorders                                   | Hypotension, flushing  |                          |
| Respiratory, thoracic and mediastinal disorders      |                        | Chest pain, dyspnoea     |
| Gastrointestinal disorders                           | Nausea, abdominal pain | Metallic taste, vomiting |
| Skin and subcutaneous tissue disorders               | Pruritus               |                          |
| Musculoskeletal and connective tissue disorders      |                        | Back pain                |
| General disorders and administration site conditions | Feeling of warmth      | Chills, pain             |

Sweating and tachycardia were reported as sporadic cases.

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions.

4.9 Overdose

Overdose of carbetocin may produce uterine hyperactivity whether or not due to hypersensitivity to this agent. Hyperstimulation with strong (hypertonic) or prolonged (tetanic) contractions resulting from oxytocin overdose can lead to uterine rupture or postpartum haemorrhage. In severe cases, overdose of oxytocin may lead to hyponatraemia and water intoxication, especially when associated with excessive concomitant fluid intake. As carbetocin is an analogue of oxytocin, the possibility of a similar event cannot be excluded. Treatment of overdose of carbetocin consists of symptomatic and supportive therapy. When signs or symptoms of overdose occur, oxygen should be given to the mother. In cases of water intoxication it is essential to restrict fluid intake, promote diuresis, correct electrolyte imbalance, and control convulsions that may eventually occur.

5 PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: Oxytocin and analogues, ATC code: H01BB03

The pharmacological and clinical properties of carbetocin are those of a long-acting oxytocin agonist.

Like oxytocin, carbetocin selectively binds to oxytocin receptors in the smooth muscle of the uterus, stimulates rhythmic contractions of the uterus, increases the frequency of existing contractions, and raises the tone of the uterus musculature.

On the postpartum uterus, carbetocin is capable of increasing the rate and force of spontaneous uterine contractions. The onset of uterine contraction following carbetocin is rapid, with a firm contraction being obtained within 2 minutes.

A single 100 micrograms intravenous dose of carbetocin administered after the delivery of the infant is sufficient to maintain adequate uterine contraction that prevents uterine atony and excessive bleeding comparable with an oxytocin infusion lasting for several hours.

5.2 Pharmacokinetic properties

Carbetocin shows a biphasic elimination after intravenous administration with linear pharmacokinetics in the dose range of 400 to 800 micrograms. The terminal elimination half-life is approximately 40 minutes. Renal clearance of the unchanged form is low, with< 1 % of the injected dose excreted unchanged by the kidney. Plasma carbetocin concentrations were detectable by 15 minutes and peaked at a maximum of 1035 ± 218 pg/ml within 60 minutes. Peak concentrations in milk were approximately 56 times lower than in plasma at 120 minutes.

6 PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Mannitol  
Acetic acid  
Sodium hydroxide (for pH adjustment)  
Water for injection

6.2 Incompatibilities

In the absence of compatibility studies, this medicinal product must not be mixed with other medicinal products.

6.3 Shelf life

24 months.

Shelf life after the first opening of the container

After the first opening of the vial, the solution should be used immediately.

6.4 Special precautions for storage

Store below 30 °C

Store in the original package in order to protect from light. Do not freeze. After the first opening of the vial, the solution should be used immediately.

6.5 Nature and contents of container

Type I colourless glass vial (2R) closed with grey bromobutyl rubber stopper and aluminium flip-off cap with plastic button containing 1 ml of solution for injection. Packs of 5 vials.

6.6 Special precautions for disposal and other handling

Only clear solutions free of visible particles should be used. Pharmldea Carbetocin is intended for intravenous or intramuscular use only. Any unused product or waste material should be disposed of in accordance with local requirements.

7 PRODUCT REGISTRATION HOLDER

Eucogen Pharma Sdn. Bhd.  
No 6A, Jalan Sungai Burung U32/U,  
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Brunei Darussalam Product Licence Holder:  
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8 MANUFACTURER

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9 DATE OF REVISION OF THE TEXT

March 2026